Basics of Intensive care – A daily checklist

General

Feeding

- Attempt enteral (NG) feeding in all patients, starting at 30ml/hr and gradually increasing to approx. 1ml/kg/hr predicted body weight.
- If gastric aspirates are high, start Metoclopramide 10mg IV tds and Erythromycin 75mg IV tds as prokinetics. Stop when have been absorbing feed for >24 hours.

Analgesia - All patients need some sort of analgesia. Use regular paracetamol ng or iv in nearly all, plus parenteral opioid as part of analgosedative regimen

Sedation

- Aim for sedation score (RASS) of -2 which means rousable to voice but drops off to sleep if no intervention.
- If deeper sedation needed consider early neuromuscular blockade with infusion of Atracurium.
- Consider need for daily sedation hold (in general if otherwise stable and oxygen requirement is moderate). If appropriate, all sedation should turned off when safe to do so. If re-sedation is required restart at half the previous sedation rate.
- Standard sedation regimens are Propofol/alfentanil, or morphine/midazolam

Thromboprophylaxis

- All patients should receive prophylactic Dalteparin 5000 units once daily unless COMPELLING reason not to.
- >120kg use bd Dalteparin 5000 units.
- Reduce in renal impairment (GFR<30) to 2500u od. Hold if thrombocytopaenic.
- Ideally use either calf pumps if asleep or TEDS if awake in addition.

Head up - Supine patients should be nursed at least 30 degrees head up.

Glucose control

- Aim for glucose of 4-11mmol/l.
- Use insulin Actrapid as IV infusion [1 unit/ ml] to maintain glucose within this range.
- **DO NOT USE** insulin without some kind of calorie source.

Fluid Balance

- In general, aim for as neutral a fluid balance as possible. ICU patients often end up very fluid overloaded because of feed, IV drugs and infusions. For patients with COVID-19, as dry as you dare.
- Strict attention to this and use of small doses of IV Furosemide (eg 10-20mg iv) will lead to a better outcome for ventilated patients.
- Non shocked patients do not need infusions of crystalloid. For boluses use Hartmann's 250ml.

Ulcer prophylaxis

- None if patients fully fed and absorbing.
- Ranitidine 50mg IV tds if not.
- PPI e.g. omeprazole if usual drug prior to admission.

Lines

- Arterial lines are often used for a long time (>10 days) if working well and look clean.
- In general, remove peripheral lines if looking red.
- Inspect lines site and duration daily.

- Remove central lines as soon as possible. No routine changes are advised, but consider new lines if new fever or vasopressor requirement, or if CVP been in for 8 days or more.

Relatives - Daily update by phone at least. All patients will have a password.

Pressure areas - Ask every day about pressure areas.

Bowels

- Constipation is very common in ICU patients.
- Use 1 or more of Senna 15mg bd , Laxido 1 sachet bd, Sodium Docusate 200mg bd prescribe early .
- Diarrhoea also common. Ensure it is not overflow, and send sample for CDiff
- Do not use Loperamide unless definitely non-infectious.

Organ Specific

Respiratory

- Aim for as low an FiO2 as possible to maintain PaO2 >8kPa.
- Standard initial suggested ventilator settings TV 6ml/kg predicted body weight, RR 20 at least, PEEP 5cms H2O. If FiO2 >0.4, use 10cms PEEP. Aim for sats 92%
- Keep plateau pressure <30cms H2O, and don't worry too much about PaCO2.
  Speak to ICU Consultant if deviations from this are necessary.

Cardiovascular

- Assess adequacy of cardiac output using peripheral perfusion, tachycardia and lactate, as well as urine output.
- Cool peripheries mean only that cardiac output is not adequate – DOES NOT NECESSARILY mean that the patient is under filled. If in doubt give judicious (250ml) fluid bolus
- INOTROPES – the vasopressor/inotrope of choice is noradrenaline, which must be administered through central line. Metaraminol is useful as a short term option, but should not be used long term.

Renal - Aim for urine output of 0.5ml/kg/hr, but do not overfill to chase this.

Haematological - Transfuse red cells at Hb of 70g/l. Platelets if count<20 or higher if bleeding.

Gastrointestinal and liver - Rises in LFTs especially ALT may be due to drugs.

New fever

- Full septic screen (blood cultures peripherally and through lines). Sputum and urine samples. Drain fluid for MCS. CXR.
- Consider removing lines if no other source suspected.
- Most likely sources of infection are chest and lines.

Daily bloods - FBC, U&E, CRP. LFTs and coag on admission and then twice weekly unless abnormal.

Blood gases - Twice daily only unless very unstable

CXR - Only if getting worse

Micro

- Review need for antibiotics daily. Regular microbiology input will be available
- **Change antibiotics on advice of microbiology or ICU consultant only**

ANY QUESTIONS – ask ICU Consultant. There are loads of ICU guidelines on the HuB.